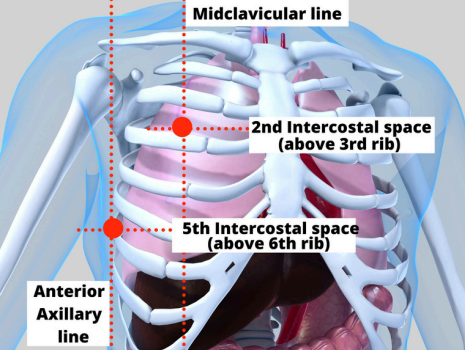


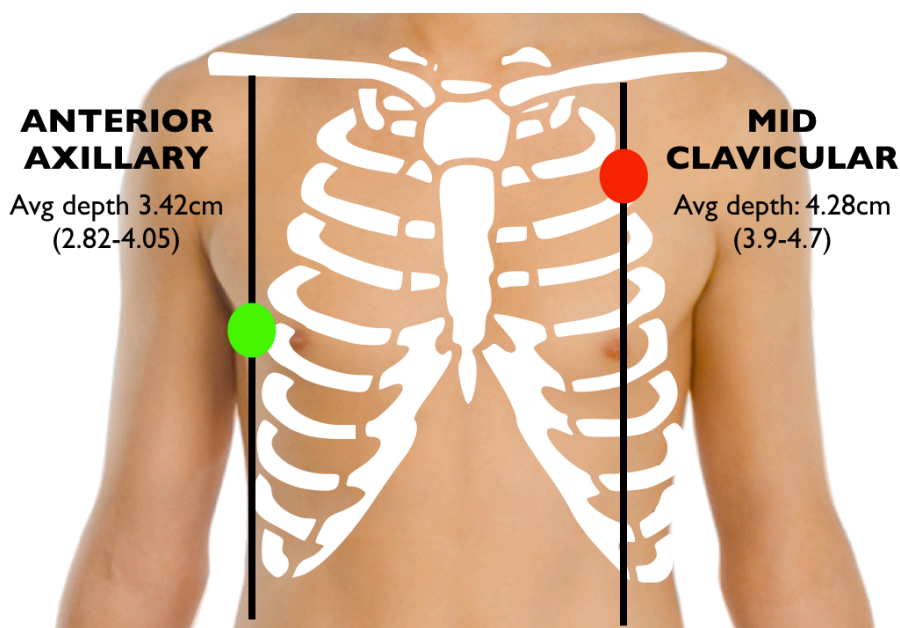


Chest Needle Thoracostomy

Chest Needle thoracostomy is performed in the setting of suspected tension pneumothorax

Indications: Pre-arrest of VSA **AND** Absent or severely diminished breath sounds on the affected side(s)

Steps to prepare	Implementation	Considerations
☐ Landmark • 4th intercostal space, anterior axillary line (preferred) • 2nd intercostal space, mid clavicular line ☐ Cleanse the needle insertion site using aseptic technique	☐ Insert needle with attached syringe into intercostal space ☐ Insert along the superior aspect of the rib with bevel facing toward rib ☐ Aspirate for air as you advance the needle ☐ Once free air is aspirated, advance needle 2mm further into the pleural space ☐ Advance the catheter over the needle to the hub & remove the needle/syringe ☐ Place needle in biohazard container ☐ Secure the catheter in place with tape ☐ Place flutter valve	☐ Success = Improvement in hemodynamic status ☐ Success = Partial or complete relief of respiratory symptoms ☐ Do not provide PPV with BVM



*** Consider chest needle thoracostomy in a cardiac arrest secondary to severe blunt or penetrating trauma prior to calling BHP for a trauma TOR ***